E/M UNIVERSITY

Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level THREE Admission H&P

99223

$193.21

E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: SOB

HPI: Patient complains of severe intermittent SOB worse when laying supine and associated with lower extremity edema.

SEVERITY TIMING MODIFYING FACTORS

ASSOCIATED SIGNS/SX

PFSH: Remarkable ischemic cardiomyopathy with EF of 20%, 20 pack year smoking history and strong family history of premature cardiovascular disease.

ROS: Constitutional: No fevers, chills, weight loss. CV: No chest pain or palpitations. GI: No N/V/D. All other systems reviewed and are negative.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

EXAM: CONSITUTIONAL: NAD, conversant. BP 166/80, HR 82, T 98.0. EYES have anicteric sclerae; no lid-lag or proptosis. ENMT exam is normal. CV: RRR, no MRGs. There is bipedal edema. RESPIRATORY: Lungs have bibasilar crackles with increased respiratory effort. GI: Abdomen is soft, NABS, no HSM. MUSCULOSKELETAL exam is normal. PSYCHIATRIC: A&OX3, normal affect.

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

Labs show a BUN of 54, creatinine of 2.8, potassium 4.2, sodium 142, HGB 12.1. First troponin 0.04. CXR was reviewed and shows pulmonary vascular congestion. EKG was reviewed and shows no diagnostic ST changes.

1 POINT FOR LABS
2 POINTS CXR REVIEWED
2 POINTS EKG REVIEWED

Impression:
1. Acute on chronic left ventricular systolic heart failure
2. New ARF, most likely cardio-renal in origin
3. Poorly controlled HTN due to volume overload

Plan:
1. BUMEX 2 mg IV Q 6H
2. DIRUIL 500 mg IV times one
3. Hold ACE inhibitor for now
4. Chem 10 in a.m.
5. DVT prophylaxis

Problem Points	7
Data Points	5
Risk	HIGH

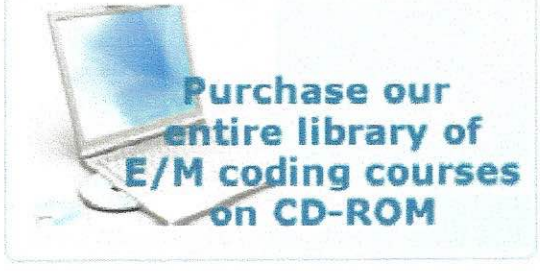
-2 PROBLEM POINTS FOR ESTABLISHED AND WORSENING OF CHF
-2 PROBLEM POINTS FOR WORSENING HTN
- 3 PROBLEM POINTS FOR ARF FOR NO FURTHER WORKUP

HIGH RISK → SEVERE EXACERBATION OF CHRONIC ILLNESS AND ACUTE RENAL FAILURE

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

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E/M Insight Case of the Week

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E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Co	Comp	Mod	50
99223	Co	Comp	High	70
(Requires 3 out of 3 key components)				

CC: "Found Down."

HPI: The patient is an unknown white male found down on the sidewalk by police. He is somnolent and unable to give any history.

FULL CREDIT GIVEN AS LONG AS YOU DOCUMENT WHY

ROS/PFSH: Could not be obtained due to the fact that the patient is unresponsive. FULL CREDIT GIVEN

History	HPI	ROS	PFSH
PF	[illegible]	[illegible]	None
EPF	Brief	[illegible]	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

PHYSICAL EXAM

Somewhat limited by the patient's inability to cooperate.
CONSTITUTIONAL: Unkempt stuperous white male.
Vitals: BP 90/55, HR 84, RR 28.
ENMT: Some mucosal pallor, poor dentition.
CARDIOVASCULAR: RRR, no MRGs, normal PMI.
RESPIRATORY: CTA, somewhat tachypneic.
GI: Abdomen soft with positive bowel sounds, no HSM.
MUSCULOSKELETAL: No digital cyanosis or edema.
PSYCH: Could not assess due to patient's somnolence.
SKIN: Warm and dry; multiple tattoos and piercings.
NEURO: Could not assess due to patient's somnolence.

FULL CREDIT GIVEN AS LONG AS YOU DOCUMENT

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

LABORATORY/RADIOGRAPHIC INFORMATION: Shows a BUN of 22 with a creatinine of 2.9, sodium 141, potassium 4.8, bicarbonate 11, calcium 8.3, phosphorous 3.9, and hemoglobin of 12.7. CXR was reviewed and showed no acute cardiopulmonary disease. CT of the head reported as negative in the chart.

1 POINT FOR LABS
2 POINTS FOR CXR REVIEWED
1 POINT FOR CT HEAD

IMPRESSION

1. Coma in patient "found down."
2. Possible toxic ingestion/poisoning.
3. Acute renal failure.
4. Severe acidosis, most likely metabolic.
5. Significant hypotension.

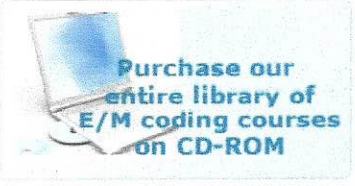
Problem Points	>4 POINTS
Risk	HIGH RISK
Data Points	4 POINTS

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

PLAN

1. Stat ABG, serum osmolality and CPK now.
2. Admit to ICU.
3. Give two amps bicarb now.
4. Bolus with 1 liter of normal saline.
5. Start D5W with 3 amps HCO3-/L at 150 cc/hr after initial bolus.
6. Repeat renal profile in four hours.

ARF → 3 POINTS
ENCEPHALOPATHY / COMA → 4 POINTS



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Except in CO,NM, OK, TX and VA where it would be down-coded to 99221 (see the red question mark by the ROS).

E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: SOB LISTED STATUS OF THREE CHRONIC ISSUES

HPI: 87 YOM NH resident with history of well controlled COPD and DM. Also has CAD s/p recent LHC with stent placement in July. Here with severe SOB.

PFSH: Remarkable dylipidemia and 50 pack year smoking history. Family history negative for premature cardiovascular disease in any first degree relatives.

ROS: Constitutional: No fevers, chills, weight loss. CV: No chest pain, orthopnea, PND. GI: No N/V/D. All other systems reviewed and are negative.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/ 3
Comp	Ext	≥ 10	3/3

EXAM: NAD, conversant. BP 164/80, HR 114, T 99.0. Eyes have anicteric sclerae; no lid-lag or proptosis. PERRLA. OP is clear with MMM. Poor dentition. Neck: Trachea midline, no masses or thyromegaly. FROM without crepitus. Heart: RRR, no MRGs. Lungs have scattered wheezes and increased respiratory effort.. Abdomen is soft, non-tender, no HSM. Extremities show 2+ bipedal edema, no cyanosis, some clubbing. Psych: A&OX3, normal affect.

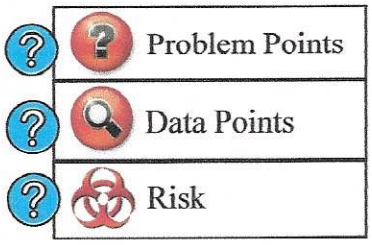
Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

Labs show a creatinine of 0.8, potassium 4.2, sodium 142, HGB 12.1. First troponin 0.04. CXR was reviewed and showed hyperinflation. EKG was reviewed and shows mild ST depression in the anterolateral leads.

1 POINT FOR LABS
2 POINTS CXR REVIEWED
2 POINTS EKG REVIEWED

Impression:
1. Acute SOB
2. Documented CAD with abnormal EKG
3. Underlying COPD
4. Stable DM

Plan:
1. Admit to telemetry
2. Heparin gtt
3. Follow enzymes
4. Nebs
5. Consider V/Q scan in a.m.
6. Sliding scale insulin

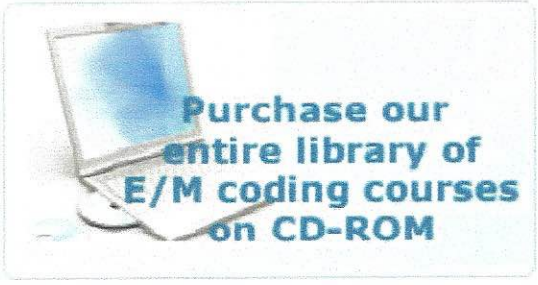


MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

HIGH RISK = NEW SOB POSES A THREAT TO LIFE OR BODILY FUNCTION

3 PROBLEM POINTS FOR SOB
1 PROBLEM POINT FOR CAD
1 PROBLEM POINT FOR COPD
1 PROBLEM POINT FOR DM





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By Peter R. Jensen, MD, CPC

Using the 1995 E/M guidelines, this E/M service is a level THREE Admission H&P

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E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: Headache

HPI: Patient complains of a constant, "pounding" headache for two days associated with blurry vision.

TIMING QUALITY LOCATION DURATION

ASSOCIATED SIGNS/SX

PFSH: No allergies. Father died at age 55 of CVA. Smokes 2 PPD.

ROS: Positive per HPI. No chest pain or SOB. No N/V. All other systems reviewed and are negative.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

Exam: NAD. BP 210/105, RR 22, HR 88. The following organ systems were normal: EYES, ENMT, CARDIOVASCULAR, RESPIRATORY, GI, NEUROLOGIC, PSYCHIATRIC.

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

Data: EKG was reviewed and showed NSR with no ST changes. CT of the head was read as NAD. Labs show normal electrolytes, CBC and normal creatinine.

2 POINTS EKG REVIEWED
1 POINT CT HEAD
1 POINT LABS

Impression: New diagnosis of HTN presenting with hypertensive urgency

Plan

1. Admit to ICU
2. LABETOLOL drip
3. Titrate dose to maintain SBP < 180
4. Start oral LABETOLOL 200 mg BID
5. Start LISINOPRIL 40 mg PO QD
6. Start AMLODIPINE 10 mg P
7. Check renal artery Doppler studies to rule out renal artery stenosis

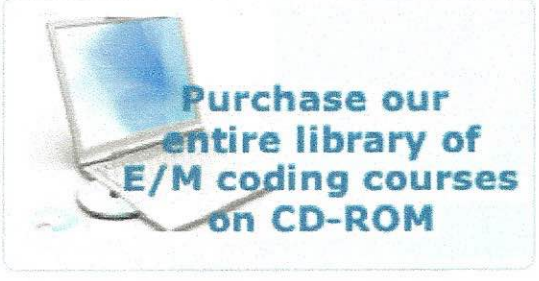
Problem Points	4
Data Points	4
Risk	HIGH

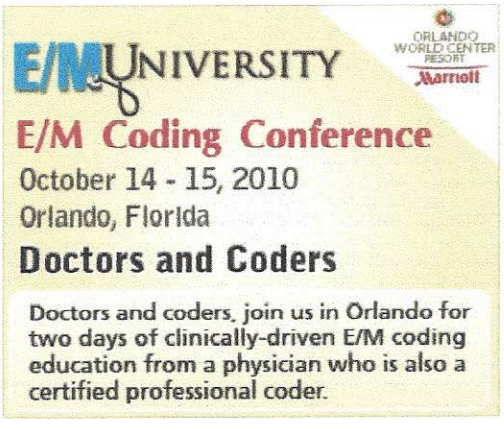
MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

4 PROBLEM POINTS - SEVERE HTN WITH WORKUP WITH RENAL US

HIGH RISK GIVEN ON LABETALOL DRIP, SEVERE HTN WHICH CAN RESULT IN THREAT TO LIFE OR BODILY FUNCTION







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E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: Chest pain.

HPI: The patient complains of intermittent, exertional sub-sternal chest pain associated with SOB.

TIMING CONTEXT ~~QUALITY~~ LOCATION ASSOCIATION SIGNS/SX

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

Allergies: Keflex

FH: Remarkable for acute MI in father at age 56.

SH: Smokes 2 PPD

REVIEW OF SYSTEMS
CV: Per HPI; negative for orthopnea or edema
Pulm: Negative for cough/hemoptysis
GI: Negative for N/V/D or abdominal pain
All other systems reviewed and are negative.

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

PHYSICAL EXAMINATION:
GENERAL: He is awake and alert in no acute distress.
VITAL SIGNS: BP 151/79, RR 20, HR 85.
EYES: Pink conjunctivae; no ptosis; PERRLA
ENMT: No rhinophyma or earlobe creases; good dentition
NECK: FROM/Supple, no JVD; no thyromegaly
LUNGS: Clear bilaterally with normal respirator effort
CARDIOVASCULAR: Regular rate and rhythm; no MRGs
ABDOMEN: Soft, non-tender, no HSM
EXTREMITIES: No peripheral edema.
SKIN: Warm and dry; well-perfursed, no rash or ulcers
PSYCH: Agitated affect, A&OX4

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

LABORATORY DATA: Laboratory information shows : Trop 0.05, BUN of 18 creatinine of 0.7, and potassium 4.1. Hemoglobin 14.1. 1 POINT FOR LABS

EKG: Reviewed. NSR, no diagnostic ST changes. 2 POINTS FOR EKG REVIEWED
CXR: Report states NAD 1 POINT

IMPRESSION
1. Chest pain consistent with USA → 4
2. HTN – 1 POINT

Problem Points	5
Data Points	4
Risk	HIGH RISK



PLAN
1. ASA given in ER
2. LMW
3. Metoprolol 25 mg PO BID
4. NTP 1" Q 6H
5. Follow cardiac enzymes per protocol
6. Consider stress echo in a.m.

4 PROBLEM POINTS FOR NEW CHEST PAIN THAT REQUIRES WORKUP WITH STRESS ECHO

ALL CHEST PAINS ARE HIGH RISK

E/M UNIVERSITY

Case of the Week

By Peter R. Jensen, MD, CPC

5/26/09

The E/M service documented is a level THREE Admission H&P

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CC: Palpitations

HPI: Patient developed intermittent palpitations (TIMING) associated with shortness of breath (ASSOCIATION SX) this morning (CONTEXT) after jogging on the treadmill (DURATION).

PFSH: Remarkable for stable HTN. No history of tobacco abuse. No premature cardiovascular disease in any first-degree relatives.

ROS: Constitutional: No fevers, chills, weight loss. CV: No chest pain, orthopnea, PND. GI: No N/V/D. All other systems reviewed and are negative.

EXAM: NAD, conversant. BP 124/80, HR 114, T 99.0. Eyes have anicteric sclerae; no lid-lag or proptosis. ENMT: WNL. Heart is irregularly irregular without MRGs. Lungs are CTA. Adomen is soft, non-tender, with no HSM. Neruological exam is non-focal. Patient is alert and oriented to person, place and time with a cordial affect.

Labs show (1 POINT) a creatinine of 0.7, potassium 4.2, sodium 142, HGB 12.1. CXR (2 POINTS) was reviewed and shows no acute disease. EKG was reviewed (2 POINTS) and shows atrial fibrillation with no ST changes.

Impression

1. New onset atrial fibrillation with rapid ventricular response. (4 POINTS HIGH RISK)
2. Well controlled HTN. (1 POINT)

Plan

1. Admit to telemetry.
2. IV HEPARIN per protocol.
3. Start CARDIZEM drip.
4. Check TSH.
5. Consult cardiology for consideration for DC cardioversion.

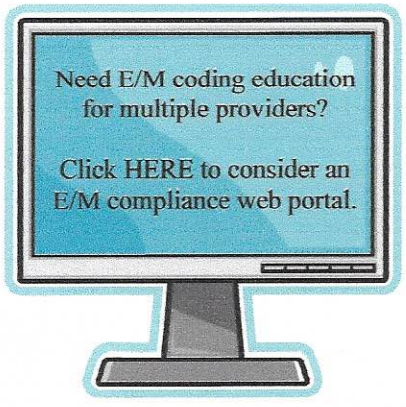
Problem Points	5
Data Points	4
Risk	HIGH RISK

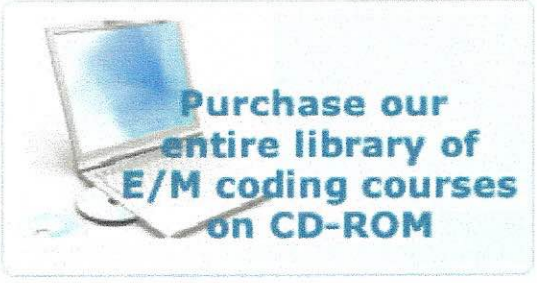
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99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

Exam	1995 Exam Rules
PF	One body area or system
EPF	2 - 4 body areas/systems
Detailed	5 - 7 body areas/systems
Comp	8 - 12 organ systems

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			





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99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: SOB

HPI: Patient is a 46 year old male who complains of constant (TIMING) and severe (SEVERITY) shortness of breath which started four hours ago (DURATION) after patient got home from the airport after a business trip. (CONTEXT)

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/ 3
Comp	Ext	≥ 10	3/3

PFSH: No allergies. Smokes about 1 PPD times 20 years. No family history of premature cardiovascular disease of primary lung disease.

ROS: Cardiovascular: Negative for palpitations or chest pain. Pulmonary: negative for hemoptysis or cough. GI: negative for N/V. All other systems reviewed and are negative.

PHYSICAL EXAM: Well-nourished male in some distress. Vitals: 160/80, 115, 28. EYES: PERRL with normal conjunctivae and lids. ENMT: Normal nasal mucosa and oropharynx. NECK: Trachea midline. No thyromegaly. LUNGS: CTA with somewhat increased respiratory effort. CARDIOVASCULAR: RRR, no MRGs. No peripheral edema. ABDOMEN: Soft and non-tender. No HSM. MUSCULOSKELETAL: Normal gait and station. Homan's sign is negative. No digital clubbing. PSYCH: A & O X 3 with appropriate affect.

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

CBC and chemistry panel is WNL. EKG was reviewed and shows sinus tachycardia, RAD and a flipped T wave in lead III. CXR was reviewed and shows no infiltrate or pulmonary vascular congestion.

1 POINT LABS
2 POINTS EKG REVIEWED
2 POINTS CXR REVIEWED

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

Assessment
1. New onset SOB consistent with acute PE → 4 POINTS
2. Sinus tachycardia

Plan
1. CT angiogram of the chest stat.
2. Check D-dimmer.
3. HEPARIN drip.

Problem Points	4
Data Points	5
Risk	– HIGH RISK - PROBABLE PE

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E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: Chest pain

TIMING LOCATION DURATION

HPI: Patient complains of intermittent sub-sternal chest pain which began yesterday and is associated with shortness of breath.

ASSOCIATED SIGNS/SX

PFSH: Remarkable for hyperlipidemia and ongoing tobacco abuse as well as a strong family history of premature cardiovascular disease.

ROS: No active chest pain now. Negative for PND, cough, nausea or vomiting. All other systems reviewed and are negative.

Exam: NAD, conversant. BP 150/80, RR 24, HR 98. EYES: normal. ENMT: normal. CV: RRR, no MRGs, trace bipedal edema. Lungs: CTA with normal respiratory effort. GI: Soft, non-tender, no HSM. Musculoskeletal: normal. Skin: normal.

Data: EKG was reviewed and showed some flipped T waves in the anterolateral leads. CXR was reviewed and shows no active disease process. Labs show normal electrolytes, CBC and normal creatinine. Troponin is 0.78

2 POINTS EKG
2 POINTS CXR
1 POINT LABS

Impression: New unstable angina.

Plan: Admit to telemetry. Follow troponins. Start ASA, beta blocker, NTP, heparin. Consult cardiology.

4 POINTS UNSTABLE ANGINA FURTHER WORKUP

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/ 3
Comp	Ext	≥ 10	3/3

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

Problem Points	4 POINTS
Data Points	5 POINTS
Risk	HIGH

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Case of the Week

By Peter R. Jensen, MD, CPC

Using the 1995 E/M guidelines, this E/M service is a level ONE Admission H&P

99221

$95.11

THIS COULD HAVE BEEN A LEVEL 3 ADMISSION H+P IF THE PHYSICAL EXAM WAS DOCUMENTED BETTER

E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: SOB

HPI: Patient complains of three days (DURATION) of intermittent (TIMING) shortness of breath which is worse with exertion (MODIFYING FACTORS) and associated with lower extremity edema (ASSOCIATED SIGNS/SX).

PFSH: Remarkable for hyperlipidemia, CAD and ischemic cardiomyopathy with EF of 25%. Non-drinker, previous smoker. FH is negative for hereditary cardiac disease.

ROS: Positive for orthopnea. No active chest pain. No PND, cough, nausea or vomiting. No fevers or chills. All other systems reviewed and are negative.

Exam: NAD, conversant. BP 160/80, RR 24, HR 98. HEENT: NC/AT. NECK: No JVD. LUNGS: Bibasilar crackles. HEART: RRR with S3 gallop. GI: Abdomen is soft with NABS. EXTREMITIES show bipedal edema.

Data: EKG was reviewed (2 POINTS) and showed some old Q waves in the anterolateral leads but no ST changes. CXR was reviewed and shows severe pulmonary vascular congestion. Labs show normal electrolytes, CBC and normal creatinine. Troponin is 0.08. (1 POINT; 2 POINTS)

Impression
1. Severe CHF exacerbation (2 POINTS)
2. Poorly controlled HTN (2 POINTS)
3. Ischemic cardiomyopathy (1 POINT)

Plan
1. Admit to telemetry
2. Monitor enzymes
3. LASIX 40 mg IV BID
4. METOLAZONE 5 mg IV now
5. Echo in the a.m.
6. Increase LISINOPRIL to 40 mg PO QD

Problem Points	5
Data Points	5
Risk	HIGH RISK

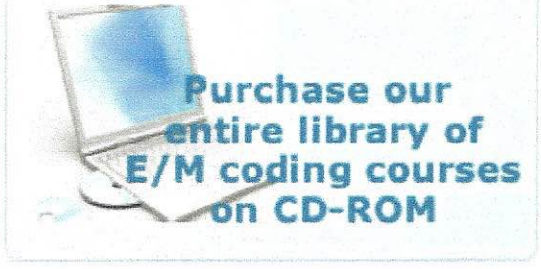
History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

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Case of the Week

By Peter R. Jensen, MD, CPC

Using the 1995 E/M guidelines, this E/M service is a level ONE Admission H&P

99221

$95.49

THIS COULD HAVE BEEN A LEVE 3 HPI ADMIT IF ROS WAS DOCUMENTED BETTER.

E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70
(Requires 3 out of 3 key components)				

CC: Shortness of breath

HPI: Patient complains of intermittent exertional shortness of breath which began yesterday and is associated with worsening lower extremity edema.

PFSH: Remarkable for hyperlipidemia, documented CAD s/p CABG in 1998. There is also a strong family history of premature cardiovascular disease in several first degree relatives and a history of tobacco abuse, although the patient quit smoking five years ago.

ROS: As reviewed in the HPI. No chest pain or cough. Otherwise non-informative.

CONSTITUTIONAL: NAD, conversant. BP 180/80, HR 72, RR 20. EYES: No ptosis. Pink conjunctivae. PERRL. ENMT: Good dentition. No pharyngeal erythema, ulcers or exudate. NECK: No masses or crepitus; trachea midline. No thyromegaly. LUNGS: CTA with normal respiratory effort. CV: Heart RRR with no MRGs; no peripheral edema. GI: Abdomen is soft and non-tender with no HSM. SKIN: Warm and dry with normal turgor; no rash, lesions or ulcers. PSYCH: A&OX3 with appropriate affect.

Data: EKG was reviewed and showed some flipped T waves in the anterolateral leads. CXR was reviewed and shows bilateral pulmonary vascular congestion. Labs show normal electrolytes, CBC and normal creatinine. Troponin is 0.88

Impression

1. New onset CHF → 4 POINTS
2. Hypertension associated with volume overload 1 POINT
3. CAD with elevated troponins 1 POINT

Plan

1. Will start intravenous loop diuretics and NTG
2. Consult cardiology
3. Echo
4. Serial troponins

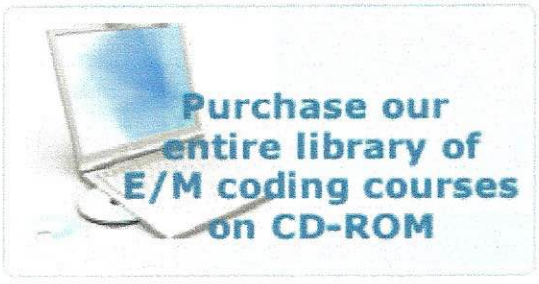
History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/ 3
Comp	Ext	≥ 10	3/3

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

Problem Points	6
Data Points	5
Risk	HIGH RISK





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Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level TWO hospital progress note.

99232

$69.63

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

CC: F/U CHF

Interval History: No new complaints offered.

Exam: NAD. 158/76, 72, 20. Lungs have bibasilar crackles. Normal respiratory effort. Heart is RRR. There is worsening symmetric bipedal edema.

NEED BETTER PHYSICAL EXAM

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥10	3/3

Chem 7 and CBC WNL. 1 POINT

Impression

1. Worsening CHF exacerbation 2 POINTS
2. Poorly controlled hypertension associated with hypervolemia 2 POINTS

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

Plan

1. Increase LASIX to 40 mg PO BID.
2. Repeat chem. 7 in a.m.

MDM	Prob. Pts	Data Pts	Risk
SF	≤1	≤1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥4	≥4	High
Requires 2/3 dimensions			

Problem Points	1
Data Points	4
Risk	→ HIGH RISK

IF YOU DOCUMENT A BETTER PHYSICAL EXAM AND STATE WORSENING OF SEVERE CHF EXACERBATION USING IV LASIX, COULD PROBABLY GET THIS TO A LEVEL 3

I'D CONSIDER THIS HIGH RISK, WE USUALLY USE IV LASIX AND SEVERE EXACERBATION OF CHF

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E/M Insight Case of the Week

The E/M service documented is a level two hospital progress note.

99232

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

CHIEF COMPLAINT: Hyponatremia.

INTERVAL HISTORY: The patient's hyponatremia has slightly worsened despite fluid restriction. HTN is controlled on current meds.

ONLY DOCUMENTED STATUS OF 2 PROBLEMS

REVIEW OF SYSTEMS: Negative for nausea or vomiting.

NEED BETTER ROS

PHYSICAL EXAM: BP is 136/76, HR 72, RR 18. No peripheral edema; appears euvolemic. NEED BETTER PHYSICAL EXAM

LABS: Sodium 124

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/ 3
Comp	Ext	≥ 10	3/3

IMPRESSION

1. Worsening hyponatremia due to SIADH 2 POINTS
2. Stable HTN 1 POINT

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

PLAN

1. Start FUROSEMIDE 20 mg PO QD
2. Repeat renal profile in a.m.

COULD PROBABLY GET MORE POINTS FOR WORKING UP WORSENING OF HYPONATREMIA WITH LABS SUCH AS ORDERING SERUM OSM, SERUM URINE OSM, URINE SODIUM

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

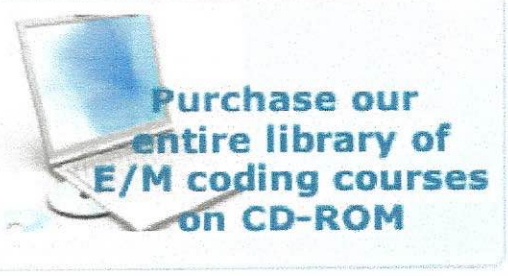
Problem Points

Data Points

Risk

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Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level TWO hospital progress note.

99232

$68.80

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/ 3
Comp	Ext	≥ 10	3/3

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

CC: F/U HTN

INTERVAL HISTORY: The patient is asleep. I didn't have the heart to wake her up. BP improving on parenteral meds, but still not optimally controlled. She remains NPO following large bowel resection.

PHYSICAL EXAM: Resting comfortably. BP 148/82, HR 72, RR 24. Lungs CTA anteriorly. Heart RRR with no MRGs. No peripheral edema. Skin is warm and dry; well perfused.

DO A BETTER PHYSICAL EXAM

Labs: Reviewed. Creatinine 1.9, potassium 4.2, sodium 126

1 POINT

Impression

1. Sub-optimally controlled HTN. 2 POINTS
2. Stable stage 3 CKD 1 POINT
3. NPO status following large bowel resection. ← I WOULD CONSIDER THIS ANOTHER PROBLEM POINT AND HIGH RISK

PLAN

1. Increase enalapril to 1.25 mg IV Q 6
2. Repeat CMP in a.m.

Problem Points	4 POINTS
Data Points	1
Risk	HIGH RISK

COULD MAKE THIS A LEVEL 3 IF BETTER PHYSICAL EXAM AND DOCUMENTING PROBLEM FOR WHY LARGE BOWEL RESECTION

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